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Decreasing Patient Falls and Staff Workload through Enhanced Safety Rounds

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Background

Falls of hospitalized patients impact safety, morbidity and mortality as well as associated costs to the hospital. It is estimated that up to 1 million patients each year experience a fall while hospitalized, with one-third resulting in injury. Many risk factors for falls in hospitalized patients have been identified with most falls occurring in the elderly, agitated, confused or with impaired judgement. These characteristics are frequently seen in patients on a Neuroscience Unit. We identified an increasing number of falls on the neuroscience unit in 2023 that continued into 2024. Analysis of the fall data revealed that 48% of these falls occurred between 3:00 am and 11:00 am. Other deficiencies found included fall signage was not appropriately used, a high volume of call lights and bed alarms caused alarm fatigue, hourly safety checks were not documented consistently. and an inadequate quantity of fall prevention supplies. Additionally, safety rounds were not purposeful, meaning staff were returning to rooms several times to address patient needs, or patient needs were going unmet.

Problem Statement and Goals

In 2023, there were a total of 41 falls on B5. As of July 2024, there were 40 falls, a rate of approximately 5.7 falls per month. In April 2024, there was a sentinel event associated with a fall. In chart audits for patients who had fallen, it was noted that hourly safety rounds were missed entirely or were inconsistently documented. Staff surveys revealed that a high volume of call lights and bed alarms were contributing to alarm fatigue. Goals:

- Decrease number of falls between 0300 – 1200 by 50% within 6 months of start date
- Decrease call light use by 25% between 0300-1200
- Rounding documentation completed with 100% compliance by RN and NA

Plan: Action Plan

Enhanced Safety Rounds (ESR)

Implement purposeful safety rounds between 0300 to 1100

- Rounds performed at 0345, 0445, 0545...1045
- RN and NA perform enhanced safety round together
- Focus of rounds: 4 P’s - Pain, personal needs, positioning, personal items
- RN/NA will use ESR accountability sheet to “sign off” that rounding was completed
- RN/NA will be assigned a rounding time in their respective POD (POD 1-3 or 4-5) by night shift charge RN

Falls Prevention Supplies

- Coordination with Supply Chain and other supply management to ensure adequate stock of:
 - Posey alarm devices
 - Posey bed and chair mats
 - Posey cords
 - Hester Davis Signs
 - Fall mats, socks and bracelets

Education

- Mandatory education assigned to RN and NA staff

Do: Corrective Actions/Interventions

Education content was developed and a module assigned to staff that focused on correct Hester Davis scoring, process for enhanced safety rounding and using the ESR tool, room set-up for increased safety, appropriate use of fall mats, and bathroom safety interventions.

Updated Fall Signage was posted on each patient room door. Signs included fall risk level as well as appropriate interventions for that fall risk level.



Enhanced Safety Round tool was created and placed in each room to be signed off by the RN and NA hourly when rounds were completed. A revised version developed to better delineate the 4 P's is shown below.

B5 Enhanced Safety Rounds Tool						
Room:	Date:					
	Pain	*Personal Needs*	*Positioning*	*Personal Items*	Pt Out of Room	Room Empty
	RN / NA Initials	RN / NA Initials	RN / NA Initials	RN / NA Initials		
0245	/	/	/	/		
0345	/	/	/	/		
0445	/	/	/	/		
0545	/	/	/	/		
0645	/	/	/	/		
0745	/	/	/	/		
0845	/	/	/	/		
0945	/	/	/	/		
1045	/	/	/	/		

Audits of the ESR tool hovered around 70% compliance despite continued reminders during shift huddles and rounds. Additional auditing by the leadership team of hourly safety rounding documentation in EHR and shift reports for communication of Hester Davis scores and fall risk interventions were completed to track progress towards the goals.

The nurse manager worked with supply chain to ensure that appropriate fall prevention supplies were stocked at increased par levels as requested.

Cipher rounding completed by leadership assisted to gauge patient satisfaction with the implementation of enhanced safety rounding.

Thank you to the B5 team for your participation
in this CUSP Project and providing a safer
environment for our patients.

Check: Evaluation of Changes

Results: Falls by Time of Day

72% decrease in falls between 0300-1100

Start of Project Jan – Aug 12 data			3 Month			6 Month		
Time of Day	Percentage	Fall rate	Time of Day	Percentage	Fall Rate	Time of Day	Percentage	Fall Rate
0700-1100	24%	9	0700-1100	0%	0	0700-1100	14%	1
1100-1500	8%	3	1100-1500	25%	4	1100-1500	0%	0
1500-1900	13%	5	1500-1900	38%	6	1500-1900	43%	3
1900-2300	21%	8	1900-2300	13%	2	1900-2300	29%	2
2300-0300	10%	4	2300-0300	5%	1	2300-0300	0%	0
0300-0700	24%	9	0300-0700	19%	3	0300-0700	14%	1
18 falls between 03-11a			3 falls between 03-11a			2 falls between 03-11a 5 falls in 6 months		

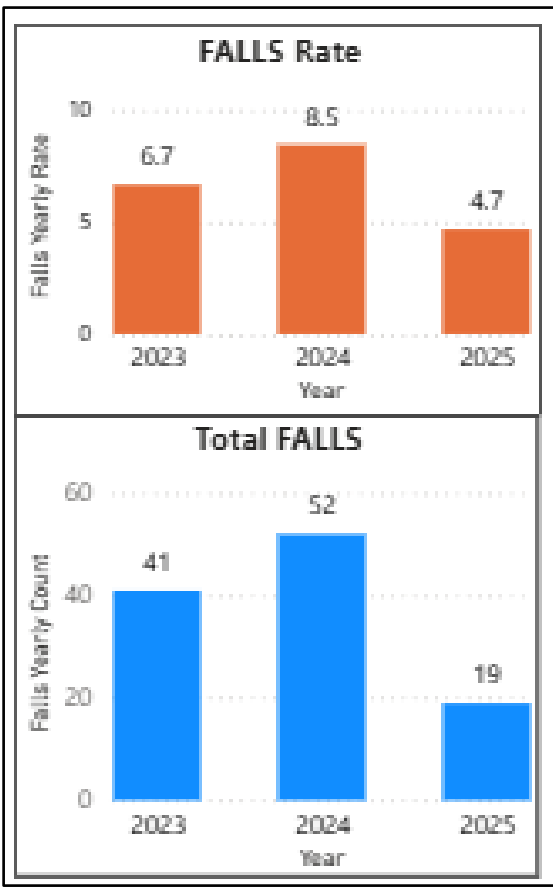
Results: Call Light Report

34% decrease in call-light usage between 0300-1100

	June 2024	October 2024
Average number of call lights per day between 0300-1100	50.35	33.4

Act: Sustain and Spread

With consistent surveillance of the fall rate, we have seen a marked reduction in the number of falls between 0300- 1100 as well as the overall number of falls in 2025. Ongoing auditing of hourly safety round documentation compliance and follow up with staff regarding completion of ESR tool continues. Consistent documentation of hourly rounding on the ESR tool and in the EHR has continued to be challenging for the staff. The staffing grid was recently updated to assist with the workload that the staff experience with this patient population. We hope to see progress in documentation with this enhancement.



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